

Comprehensive Breast Cancer Screening Guidelines

- 1.** Mammography remains the gold standard for breast cancer screening. For individuals with dense breast tissue, digital breast tomosynthesis (DBT) may be more effective and is increasingly recommended alongside traditional mammography.
- 2.** Breast ultrasound may be used as an adjunct to mammography for individuals with dense breasts or those who are at elevated risk and cannot undergo MRI screening. It enhances detection rates but may lead to increased false positives.
- 3.** MRI screening is highly sensitive and is recommended annually for high-risk individuals, including those with BRCA1/BRCA2 gene mutations, a strong family history, or prior chest radiation therapy before the age of 30.
- 4.** Risk assessment models such as the Gail Model or Tyrer-Cuzick Model should be used to estimate a woman's lifetime risk of breast cancer and guide screening recommendations.
- 5.** Clinical breast exams (CBEs) remain a part of regular health maintenance and should be performed by a trained healthcare provider, especially in individuals with symptoms or a family history of breast cancer.
- 6.** Breast self-awareness involves familiarity with one's own breasts and recognition of changes such as lumps, pain, asymmetry, nipple discharge, or skin changes. This practice should be encouraged in all individuals regardless of age.
- 7.** Genetic counseling and testing are advised for those with a family history suggestive of hereditary cancer syndromes. Positive results can influence both screening strategy and preventive measures.
- 8.** Early detection through imaging and risk stratification significantly improves treatment outcomes, reduces the need for aggressive therapies, and increases survival rates.
- 9.** Screening strategies should be tailored to individual risk profiles, incorporating genetic, lifestyle, reproductive, and environmental risk factors. One-size-fits-all recommendations may

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lead to over- or under-screening.

- 10.** Individuals with prior atypical hyperplasia or lobular carcinoma in situ (LCIS) are at increased risk and should follow enhanced screening protocols including MRI and more frequent imaging.